INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Tamoxifen · daily hormone therapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Regimen source
NCCP 00253
Version 6

Continuous daily tablets

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Tamoxifen · daily hormone therapy
National Cancer Control Programme (NCCP) 00253

How this medicine supports your treatment

After early breast cancer treatment, endocrine therapy reduces the risk of recurrence. In advanced disease, the aim is to control cancer and symptoms. Your setting determines the duration and follow-up.

HORMONE THERAPY · OESTROGEN-RECEPTOR MODULATOR

Tamoxifen

Occupies oestrogen receptors in breast-cancer cells and reduces the growth signals they receive

Oestrogen can signal through the receptor → Tamoxifen and active breakdown products occupy the receptor → Less oestrogen-driven signalling reaches the cancer cell

Tamoxifen is a selective oestrogen-receptor modulator. It blocks the effects of oestrogen in breast tissue but has different effects in some other tissues. Your liver converts it into active forms, including endoxifen. Some medicines affect that conversion, so a pharmacist should review prescriptions, supplements and over-the-counter products.

Ask for help with hot flushes, sleep, mood, sexual symptoms or side effects that make daily doses difficult. Report clot symptoms, unusual vaginal bleeding or new visual changes promptly.

The treatment being discussed

Who this information covers

Adjuvant or advanced oestrogen-receptor-positive breast cancer in pre- or postmenopausal women, within the exact NCCP eligibility criteria.

NCCP 00253 v6 · official regimen

Medicine and schedule

Take tamoxifen once daily continuously. There is no planned week off. The prescriber confirms the duration for the treatment setting. Swallow the tablet whole, with or without food. If a dose is missed or vomited, do not replace it; take the next scheduled daily dose.

NCCP 00253 v6 · official regimen

Review and duration

Swallow the tablet whole, with or without food. If a dose is missed or vomited, do not replace it; take the next scheduled daily dose. Symptoms can occur at any time; this is not a chemotherapy nadir or recovery curve. Your team agrees the timing of ongoing blood and eye checks. Heart-rhythm checks depend on your risks, symptoms and prescribed product.

NCCP 00253 v6 · official regimen

Individual risks and support

Discuss the expected benefit in your treatment setting alongside clot, womb, eye and liver risks and effects on daily life. Tell the team about previous clots, severe liver disease and all other medicines. Tamoxifen is not contraception: discuss an effective non-hormonal method during treatment and for nine months after stopping, where relevant. Absent periods do not prove infertility. Pregnancy and breastfeeding are excluded from this NCCP pathway. Do not change antidepressants, blood thinners or other regular medicines yourself.

NCCP 00253 v6 · official regimen

Why tamoxifen is offered

Taking the planned course after early breast cancer reduces the chance of the cancer returning and lowers the risk of dying from breast cancer. A longer course may bring further benefit for some people.

The size of the benefit depends on your cancer, previous treatment and remaining risk. Early-breast study results do not predict how an individual with advanced breast cancer will respond. Longer treatment also carries risks and should be reviewed with the team.

When to get help

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Get urgent help for a swollen painful leg, chest pain, new breathlessness, stroke symptoms, fainting with palpitations, swelling of the face or throat, or a severe blistering rash. Tell the team promptly about unusual vaginal bleeding, visual changes, jaundice, unusual bruising or symptoms that make treatment difficult.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Read the row for your treatment setting. Columns show order, not equal lengths of time. Agree the due window, booking team, result reviewer and whom to contact if a check is missing.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00253 v6 · official regimen · Irish Tamox (Rowex) SmPC · 30 June 2026 · Pan-Asian adapted ESMO early breast cancer guidelines · Pan-Asian adapted ESMO metastatic breast cancer guidelines · NICE NG 101: early and locally advanced breast cancer · ESO-ESMO ABC 5 consensus: tumour-marker interpretation

THE WHOLE TREATMENT JOURNEY

Tamoxifen · daily hormone therapy: treatment and checks

Your treatment pattern

Continuous daily tablets

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

ER-positive early or advanced breast cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Treatment plan NCCP 00253	Confirm indication and prescription	Start the agreed daily / injection schedule	Continue unless prescriber changes the plan	Review duration, benefit and tolerability	Agree continuation, switch or completion
Bloods / symptom review NCCP 00253	Blood count, kidney and liver profile; discuss calcium check	Review symptoms, medicines and product-specific blood plan	Irish Tamox: regular blood count, liver and calcium checks; agree frequency	Review adherence, symptoms and results	Follow unresolved abnormalities
Heart-rhythm checks NCCP 00253	electrocardiogram (ECG) and electrolytes if heart-rhythm risk indicates	Repeat according to risk, symptoms and product	Prompt review for palpitations, fainting or dizziness	No routine echo timetable for tamoxifen alone	Follow any identified abnormality

Care / check	Before starting	First weeks	During treatment	Longer-term review	Change / completion
Bone density / dual-energy X-ray absorptiometry (bone density) (DXA) NCCP 00253	Assess fracture risk and menopausal state	DXA only when indicated	No cycle-based DXA appointments	Repeat only according to the bone-risk plan	Continue bone care according to risk
Early-breast surveillance NCCP 00253	Confirm early / curative setting	Set first follow-up mammogram date	Assess new symptoms promptly	Annual mammography of remaining breast tissue	No routine computed tomography (CT) / markers if asymptomatic
Advanced-disease response NCCP 00253	Document disease sites / baseline assessment	New symptoms can bring assessment forward	Generally every 2–4 months; individualise	Adapt interval and imaging to disease / benefit	Markers supplement, never replace, assessment
Gynaecological / eye review NCCP 00253	Irish Tamox recommends an eye examination before treatment	Prompt assessment of new bleeding or visual symptoms	Eye review if symptoms; no routine screening ultrasound or biopsy	NCCP annual gynaecological examination	Follow symptoms and any abnormal findings
Early: radiotherapy ESMO_EBC2024	Review surgery / pathology	Plan radiotherapy if indicated	Coordinate endocrine timing	Follow the early-breast plan	Review late effects when relevant

What each check is for

1 Confirm the plan

Before starting or changing treatment

Adjuvant or advanced oestrogen-receptor-positive breast cancer in pre- or postmenopausal women, within the exact NCCP eligibility criteria.

Ask / act: Confirm treatment setting, medicines, menopause and the planned review.

2 Checks and symptoms

At the agreed review points

Before treatment, check blood count, kidney and liver function, clot history, medicines and pregnancy possibility. The current Irish Tamox product information also recommends an eye examination before treatment and regular blood-count, liver and calcium checks; the team agrees their timing for your product. ECG and electrolyte checks depend on heart-rhythm risks or symptoms. NCCP recommends an annual gynaecological examination. Report unusual vaginal bleeding or visual changes promptly.

Ask / act: Agree who books tests, reviews results and responds to new symptoms.

3

Review ongoing benefit

At setting-specific decision points

After early breast cancer treatment, endocrine therapy reduces the risk of recurrence. In advanced disease, the aim is to control cancer and symptoms. Your setting determines the duration and follow-up.

Ask / **act:** Review duration, alternatives and patient priorities; do not stop or extend treatment automatically.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Checks throughout treatment

Your team arranges tests and explains the results. A planned check is not confirmation that it has been completed.

Treatment-specific checks

Before treatment, check blood count, kidney and liver function, clot history, medicines and pregnancy possibility. The current Irish Tamox product information also recommends an eye examination before treatment and regular blood-count, liver and calcium checks; the team agrees their timing for your product. ECG and electrolyte checks depend on heart-rhythm risks or symptoms. NCCP recommends an annual gynaecological examination. Report unusual vaginal bleeding or visual changes promptly.

NCCP 00253 v6 · official regimen

Heart checks

ECG or other heart tests depend on symptoms and personal risk; this medicine alone does not require a routine HER2-style echocardiogram schedule.

NCCP 00253 v6 · official regimen

Bone health

Tamoxifen may lower bone density before menopause and can help maintain it after menopause. Your own fracture risks and menopausal state guide whether a bone-density scan is needed.

Irish Tamox (Rowex) SmPC · 30 June 2026

Your setting changes the pathway

Choose the pathway that your team has confirmed; these are not all extra treatments you must have.

Early-breast follow-up

After curative treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation or the opposite breast after unilateral mastectomy. The mastectomy side is not routinely mammographed. Routine CT, bone scans and tumour markers are not recommended for asymptomatic early-breast surveillance. New symptoms need assessment between appointments.

Pan-Asian adapted ESMO early breast cancer guidelines · NICE NG 101: early and locally advanced breast cancer

Surgery, chemotherapy and radiotherapy

In an early-breast pathway, chemotherapy, if indicated, precedes endocrine treatment. Surgery and its pathology guide adjuvant choices. Postoperative radiotherapy is usually delivered after recovery and any adjuvant chemotherapy; aromatase inhibitors may overlap radiotherapy under the agreed plan. Tamoxifen timing around radiotherapy is agreed by the treating teams. None of these additional treatments is automatically required by an endocrine prescription.



Advanced-disease scans

For advanced breast cancer, response assessment is generally every two to four months, adapted to disease pace, sites, symptoms and treatment. Indolent disease may be reviewed less often; new symptoms can prompt earlier scans. The team chooses the imaging method; this is not a universal three-month CT chest/abdomen/pelvis order.

Tumour markers

Tumour markers may supplement imaging and clinical assessment in advanced disease if they are informative, especially if elevated at baseline. They do not replace scans, and a marker change alone should not determine treatment failure. There is no universal fixed marker timetable.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your endocrine treatment and review milestones

No dates or personal details are needed. Daily treatment continues between reviews; a review is not an automatic stop or restart instruction.

Treatment or check	Timing in this view
Daily medicine	**First weeks → During treatment → Longer-term review → Change / completion** 20 mg orally once daily · continuous unless changed by prescriber
Before the first dose	**Before starting** Eligibility, baseline bloods and product-specific eye and risk checks
Symptoms and tolerability	**First weeks → During treatment → Longer-term review → Change / completion** May persist or change; report troublesome symptoms early
Planned reviews	**During treatment → Longer-term review → Change / completion** Benefit, adherence, bone health and setting-specific follow-up

Swallow the tablet whole, with or without food. If a dose is missed or vomited, do not replace it; take the next scheduled daily dose.

Your team agrees the timing of ongoing blood and eye checks. Heart-rhythm checks depend on your risks, symptoms and prescribed product.

Symptoms can occur at any time; this is not a chemotherapy nadir or recovery curve.

> Columns show sequence, not equal durations. These are reference milestones, not booked appointments. Symptom bands do not predict severity or a safe time to ignore symptoms.

Practical support

Help with daily treatment

Ask early for help with flushes, sweats, sleep, mood, vaginal or sexual symptoms, nausea or fatigue. A difficult side effect matters even if it is not medically severe. The team can discuss symptom treatment and the endocrine plan.

Support medicines

This oral hormone treatment does not require routine steroid premedication, injections to raise white cells or a standard anti-sickness course. Symptom medicines are chosen individually and checked for interactions.

Bloods, eyes and reviews

Agree who arranges the recommended eye and blood checks, their due windows and who reviews the results. Report bleeding or visual changes between appointments.

Bone and activity

The effect on bone differs before and after menopause. Discuss your own fracture risks and whether a bone-density test is appropriate; activity and bone care should suit your circumstances.

Taking tablets / attending injections

Practical treatment details

Make one daily dose easier

Ask the pharmacy for help with supply or swallowing difficulties.

Surgery, immobility and clot risk

Tell the surgical and oncology teams that you take tamoxifen. They decide whether any temporary interruption is needed; there is no automatic fixed interruption for everyone.



Review all medicines

Some reduce formation of active endoxifen or add bleeding or heart-rhythm risks.

Questions to ask

When are bloods, ECGs or echoes needed?

Before treatment, check blood count, kidney and liver function, clot history, medicines and pregnancy possibility. The current Irish Tamox product information also recommends an eye examination before treatment and regular blood-count, liver and calcium checks; the team agrees their timing for your product. ECG and electrolyte checks depend on heart-rhythm risks or symptoms. NCCP recommends an annual gynaecological examination. Report unusual vaginal bleeding or visual changes promptly. There is no routine echo schedule for tamoxifen alone.

Which scans apply to me?

After curative treatment, annual mammography is generally used for remaining breast tissue: both breasts after conservation or the opposite breast after unilateral mastectomy. The mastectomy side is not routinely mammographed. Routine CT, bone scans and tumour markers are not recommended for asymptomatic early-breast surveillance. New symptoms need assessment between appointments. For advanced breast cancer, response assessment is generally every two to four months, adapted to disease

pace, sites, symptoms and treatment. Indolent disease may be reviewed less often; new symptoms can prompt earlier scans. The team chooses the imaging method; this is not a universal three-month CT chest/abdomen/pelvis order.

What is my bone-health plan?

Discuss your menopausal state and personal fracture risks. Tamoxifen can affect bone differently before and after menopause. Your team will advise whether a bone-density test is needed.

How long will treatment continue?

For early breast cancer, discuss the initial endocrine course and a later decision about extending or changing treatment; some patients continue endocrine therapy to 10 years. Advanced treatment continues according to benefit and tolerance. A five-year review is not an automatic instruction to stop.

Do I need a port or home pump?

No port or home infusion pump is needed for this medicine itself. Any separately prescribed medicine has its own access requirements.

Can I keep taking my other medicines?

Give the team a complete list, including antidepressants, blood thinners, hormone preparations and supplements. A pharmacist will check whether changes are needed. Do not stop a regular medicine yourself.

What if side effects make me want to stop?

Contact the team early. Help with symptoms or a review of the endocrine plan may make treatment easier. Do not wait until you miss several doses to ask for support.

Explain the plan back: What is your treatment schedule, which checks apply to your setting, and who books and reviews them?

Take tamoxifen once daily continuously. There is no planned week off. The prescriber confirms the duration for the treatment setting.

> Clinician supportive-care discussion draft. This is not an issued prescription.

Minimal emetogenic risk

No routine prophylactic antiemetic, corticosteroid premedication or granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) order is populated for this endocrine monotherapy. Review persistent nausea or other symptoms and prescribe individually if needed.

Medicine-specific support

- Ask early for help with flushes, sweats, sleep, mood, vaginal or sexual symptoms, nausea or fatigue. A difficult side effect matters even if it is not medically severe. The team can discuss symptom treatment and the endocrine plan.
- This oral hormone treatment does not require routine steroid premedication, injections to raise white cells or a standard anti-sickness course. Symptom medicines are chosen individually and checked for interactions.
- Agree who arranges the recommended eye and blood checks, their due windows and who reviews the results. Report bleeding or visual changes between appointments.

- The effect on bone differs before and after menopause. Discuss your own fracture risks and whether a bone-density test is appropriate; activity and bone care should suit your circumstances.
- Discuss the expected benefit in your treatment setting alongside clot, womb, eye and liver risks and effects on daily life. Tell the team about previous clots, severe liver disease and all other medicines. Tamoxifen is not contraception: discuss an effective non-hormonal method during treatment and for nine months after stopping, where relevant. Absent periods do not prove infertility. Pregnancy and breastfeeding are excluded from this NCCP pathway. Do not change antidepressants, blood thinners or other regular medicines yourself.
- NCCP: no expected renal/dialysis adjustment; no mild/moderate hepatic adjustment; severe hepatic impairment not recommended. Do not invent creatinine, aspartate aminotransferase (AST), bilirubin or generic toxicity dose ladders.

Individual plan

Prescriber / date: _____ Pharmacy review: _____

Verify allergies, interactions, relevant organ function, treatment setting and the exact medicine before prescribing.

NCCP 00253 v6 · Monitoring and treatment map

Tamoxifen · daily hormone therapy

Take tamoxifen once daily continuously. There is no planned week off. The prescriber confirms the duration for the treatment setting.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hot flushes / sleep

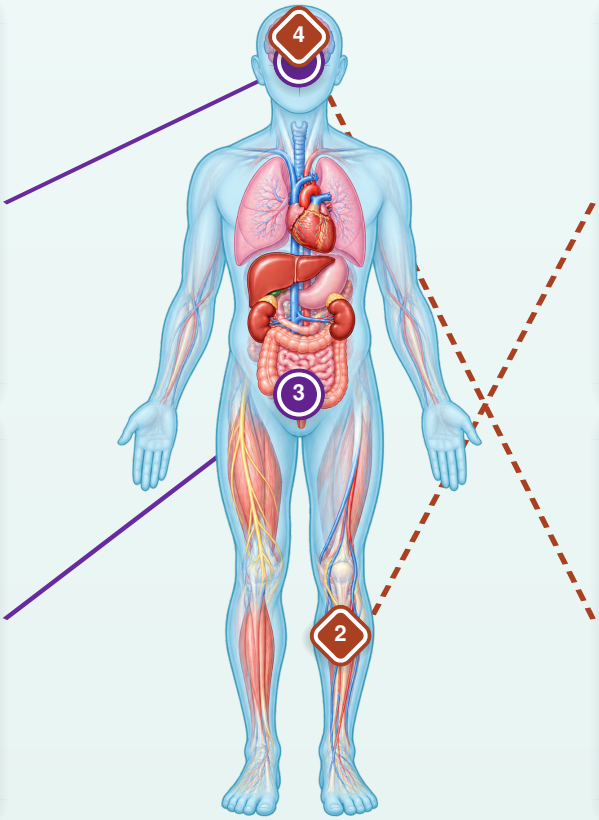
Tamoxifen

Support is available for flushes, sweats and sleep disruption.

2 Clot symptoms

Tamoxifen

New one-sided leg swelling needs urgent advice; chest pain or breathlessness may be an emergency.



3 Vaginal bleeding

Tamoxifen

Report unusual bleeding promptly; do not wait for the annual review.

4 Vision

Tamoxifen

Report blurred vision, eye symptoms or other new visual changes.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Get urgent help for a swollen painful leg, chest pain, new breathlessness, stroke symptoms, fainting with palpitations, swelling of the face or throat, or a severe blistering rash. Tell the team promptly about unusual vaginal bleeding, visual changes, jaundice, unusual bruising or symptoms that make treatment difficult. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00253 v6 · official regimen**
   https://healthservice.hse.ie/documents/6357/253_Tamoxifen_Monotherapy.pdf
2. **Irish Tamox (Rowex) SmPC · 30 June 2026**
   https://assets.hpra.ie/products/Human/19894/Licence_PA0711-022-002_30062026111821.pdf
3. **Pan-Asian adapted ESMO early breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC11145753/
4. **Pan-Asian adapted ESMO metastatic breast cancer guidelines**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC10186487/
5. **NICE NG 101: early and locally advanced breast cancer**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
6. **ESO-ESMO ABC 5 consensus: tumour-marker interpretation**
   https://pmc.ncbi.nlm.nih.gov/articles/PMC7510449/
7. **EBCTCG: five years of adjuvant tamoxifen versus no tamoxifen**
   https://pubmed.ncbi.nlm.nih.gov/21802721/
8. **ATLAS: continuing tamoxifen to ten years versus stopping at five years**
   https://pubmed.ncbi.nlm.nih.gov/23219286/
9. **ACOG Committee Opinion 601: tamoxifen and uterine cancer**
   https://pubmed.ncbi.nlm.nih.gov/24848920/
10. **Irish Tamoxifen (Viatris) SmPC · 19 May 2026**
    https://assets.hpra.ie/products/Human/17076/Licence_PA23266-052-001_19052026131657.pdf
11. **ATLAS 2013 primary report**
    https://pmc.ncbi.nlm.nih.gov/articles/PMC3596060/
12. **aTTom 2013 ASCO meeting abstract**
    https://ascopubs.org/doi/10.1200/jco.2013.31.18_suppl.5